



FACULTY OF COMPUTING AND INFORMATION SYSTEMS
CSDS 336 - FUNDAMENTALS OF MACHINE LEARNING

BRAIN TUMOR DETECTION MODEL ANALYSIS REPORT

REPORTED BY

JESSE AARON QUARTEY 1702306956

OF PROGRAMME BSC DATA SCIENCE AND ANALYTICS

LEVEL 300

GROUP D

TABLE OF CONTENTS

LIST OF TABLES.....	3
LIST OF FIGURES	4
1 INTRODUCTION.....	5
1.1 BACKGROUND OF STUDY	5
1.2 PROBLEM STATEMENT	5
1.3 AIM	6
1.4 OBJECTIVES	6
1.5 SIGNIFICANCE OF PROJECT	6
1.6 PRELIMINARY LITERATURE REVIEW	6
1.7 METHODOLOGY	7
1.7.1 DATA ACQUISITION	7
1.7.2 DATA PREPROCESSING	7
1.7.3 MODEL SELECTION AND DEVELOPMENT	8
1.7.4 HYPERPARAMETER TUNING	8
1.7.5 EVALUATION AND INFERENCE	10
Comparative Analysis	14
Inference	14
1.8 REFERNCES	15

LIST OF TABLES

Table 1. Structure of all models	8
Table 2. Training Evaluation Metrics.....	10
Table 2.1 Validation Evaluation Metrics.....	10

LIST OF FIGURES

Figure 1. Diagram of Conceptual framework	7
Figure 2. Model 1 Validation Curve	11
Figure 2.1 Model 2 Validation Curve	11
Figure 2.2 Model 3 Validation Curve	12
Figure 3. Model 1 ROC Curve.....	12
Figure 3.1 Model 2 ROC Curve.....	13
Figure 3.2 Model 3 ROC Curve.....	13

1 INTRODUCTION

Brain tumors are abnormal growths of cells in the brain which can be life-threatening if not diagnosed early. The World Health Organization claims that around 400,000 individuals around the world are diagnosed with a brain tumor (Abdalla et al., 2018). In the US, it has been estimated that around 86,970 new cases of brain tumors will be diagnosed in 2019 (Abdalla et al., 2018). Automated detection using medical imaging can assist radiologists in faster diagnosis. This project explores using convolutional neural networks (CNNs) to classify brain MRI images into pituitary tumor and no-tumor categories.

Although the initial studies on the use of machine learning in predicting brain tumors show the potential of this technology, they also highlight the limitations of this approach like efficiency of model and model accuracy ((Hossain et al.,2019), (Brindha et al.,2021)). By conducting a follow-up study, we can hopefully address these issues and contribute to the advancement of this technology. The implementation of machine learning in brain tumor treatment could represent a massive breakthrough in patient outcome prediction and could potentially help in saving lives (Sharma et al.,2022).

1.1 BACKGROUND OF STUDY

Brain MRI images are commonly used in diagnosing brain tumors. Traditional diagnosis requires manual inspection by radiologists, which, based on precedence, is time-consuming and subject to human error.

Machine learning, especially deep learning with CNNs, has shown potential in accurately classifying medical images. This new found technology, along with its methodologies and problem-solving algorithms, has proven to boost efficiency and accuracy of early detection of brain tumors.

1.2 PROBLEM STATEMENT

The problem identified here is how manual inspection of MRI scans can be time-intensive and prone to errors. Early detection of tumor is very crucial to saving the lives of the infected patients. Currently, most methods rely on CATs and MRI to find brain cancers, which have

limitations in terms of accuracy (Sharma et al.,2022). Furthermore, these tools do not provide enough precision as needed isn't critical (Brindhya, P and other writers., 2021). Furthermore, they are very likely to bring subjectivity into their analytical process, hence highlighting the need for more precise and neutral methods (Sharma et al., 2022).

The challenge is to build a reliable machine learning model that can automatically classify brain MRI images with high accuracy and generalization.

1.3 AIM

The aim is to develop and compare multiple CNN models from scratch for classifying brain MRI images into pituitary tumor and no-tumor classes. This project is focused on developing a method that uses deep learning and MRI data. This involves training neural networks on small sets of data, such as images of brain tumors, in order to study what the best model is for such dataset.

1.4 OBJECTIVES

The objectives of this project are;

1. Build three CNN models from scratch and compare their performance.
2. Evaluate models using AUC, precision, recall and accuracy. As well as a validation curve.

1.5 SIGNIFICANCE OF PROJECT

This project demonstrates the use of deep learning in medical image classification by highlighting the effectiveness of CNNs in assisting radiologists. It also provides insight into the trade-offs between model complexity, regularization, and dataset size, which is useful for future ML projects in healthcare.

1.6 PRELIMINARY LITERATURE REVIEW

Previous studies have shown that CNNs achieve high accuracy in medical image classification tasks such as breast cancer detection, lung nodule detection, and brain tumor detection. Data augmentation and hyperparameter tuning improve generalization, but small datasets can limit the effectiveness of deeper models.

1.7 METHODOLOGY

1. Dataset Acquisition
2. Data Preprocessing
3. Model Selection and Development
4. Training and Hyperparameter Tuning
5. Evaluation
6. Comparative Analysis and Inference

Conceptual Framework

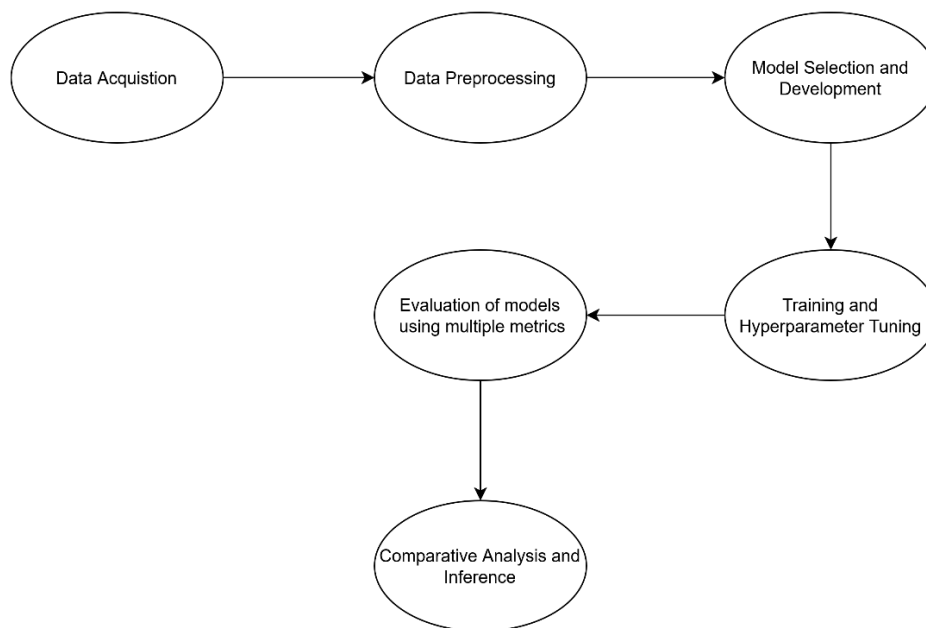


Figure 1. Diagram of Conceptual framework

1.7.1 DATASET ACQUISITION

The dataset used for this project was obtained from Kaggle, a public repository of pre-gathered and structured datasets.

Specifically, the Brain Tumor Classification MRI dataset was sourced from Kaggle at: <https://www.kaggle.com/datasets/sartajbhuvaji/brain-tumor-classification-mri/>

The dataset has four classes namely:

- glioma_tumor
- meningioma_tumor
- pituitary_tumor

- no_tumor

As indicated by their names, the classes are self-descriptive. However, in this experiment, only the **pituitary_tumor** and **no_tumor** classes were selected.

This decision was made due to the limited size of the dataset, which may not be sufficient for effective generalization in a multi-class classification setting. By reducing the task to binary classification, the models are able to learn more discriminative features and achieve more reliable performance.

1.7.2 DATA PREPROCESSING

The preprocessing technique used here was image augmentation for increasing the size of the dataset for better generalization. This included:

- Rescaling
- Flipping
- Resizing
- Zooming
- Rotation

The intensity of image augmentation was done differently for each model to better comparative analysis.

1.7.3 MODEL SELECTION AND DEVELOPMENT

Three models were made in this project using the Convolutional Neural Networks (CNN) algorithm. The following table shows the models with their architectural layers:

Model	Architecture	Regularization / Augmentation
Model 1 (Baseline)	Shallow CNN with single convolutional block [Conv2D(16)]	Rescaling only
Model 2 (Tuned CNN)	Moderately deep CNN with two convolutional blocks and increased feature capacity [Conv2D(16→32)]	Dropout + heavy augmentation
Model 3 (Deeper CNN)	Deeper CNN with three convolutional layers and higher representational capacity [Conv2D(16→32→32)]	Dropout + mild augmentation (resizing and rescaling only)

Table 1. Structure of all models

1.7.4 HYPERPARAMETER TUNING

Hyperparameters for Model 1

- Learning rate = 0.001
- Batch size = 8
- Epochs = 15
- Number of filters on 1st convolutional layer = 16

Hyperparameters for Model 2

- Learning rate = 0.0001
- Batch size = 8
- Epochs = 15
- Number of filters on 1st convolutional layer = 16
- Number of filters on 2nd convolutional layer = 32
- Dropout for 1st convolutional layer = 0.25
- Dropout for 2nd convolutional layer = 0.50

Hyperparameters for Model 3

- Learning rate = 0.0001
- Batch size = 8
- Epochs = 15
- Number of filters on 1st convolutional layer = 16
- Number of filters on 2nd convolutional layer = 32
- Number of filters on 3rd convolutional layer = 64
- Dropout for 2nd convolutional layer = 0.2
- Dropout for 3rd convolutional layer = 0.3

1.7.5 EVALUATION AND INFERENCE

Metric	Model 1	Model 2	Model 3
Training Accuracy	1.0000	0.9182	0.8813
Training AUC	1.0000	0.9098	0.9195
Training Precision	1.0000	0.9758	0.8723
Training Recall	1.0000	0.9647	0.9661

Table 2. Training Evaluation Metrics

Metric	Model 1	Model 2	Model 3
Validation Accuracy	0.9497	0.7318	0.6089
Validation AUC	0.9983	0.8246	0.6403
Validation Precision	1.0000	0.9333	0.5370
Validation Recall	0.8784	0.3784	0.3919

Table 2.1 Validation Evaluation Metrics

Validation Curves for Each Model

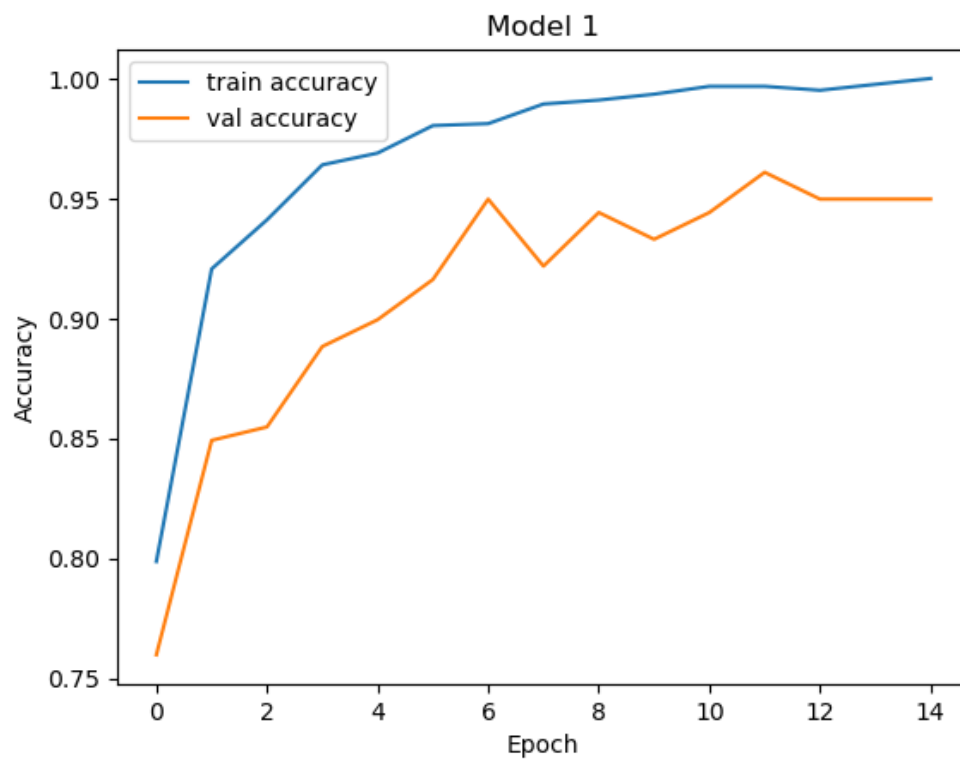


Figure 2. Model 1 Validation Curve

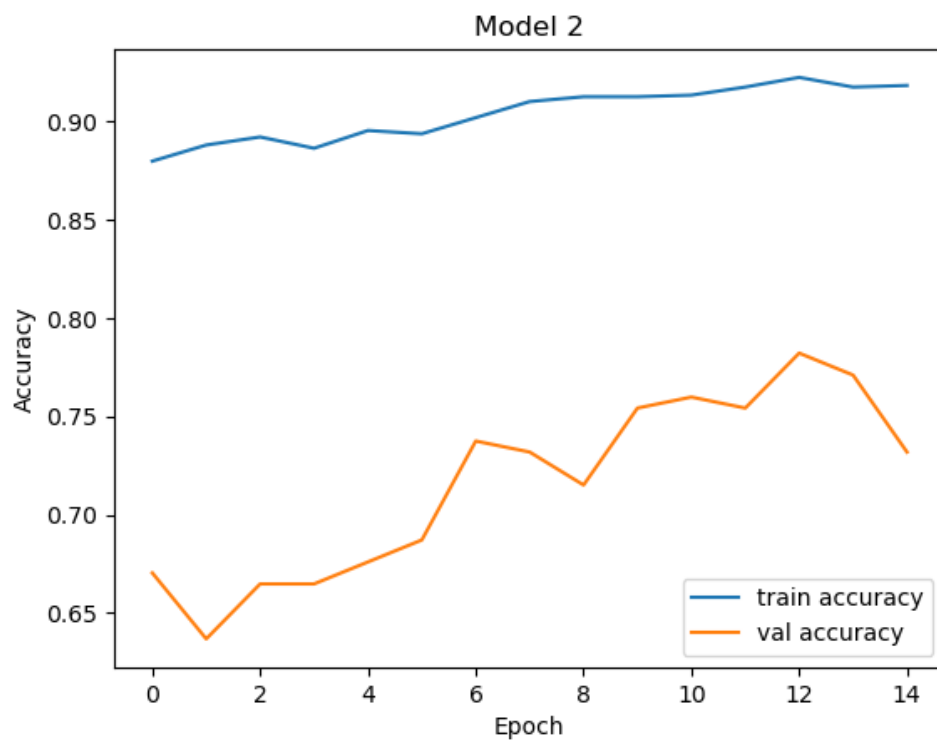


Figure 2.1 Model 2 Validation Curve

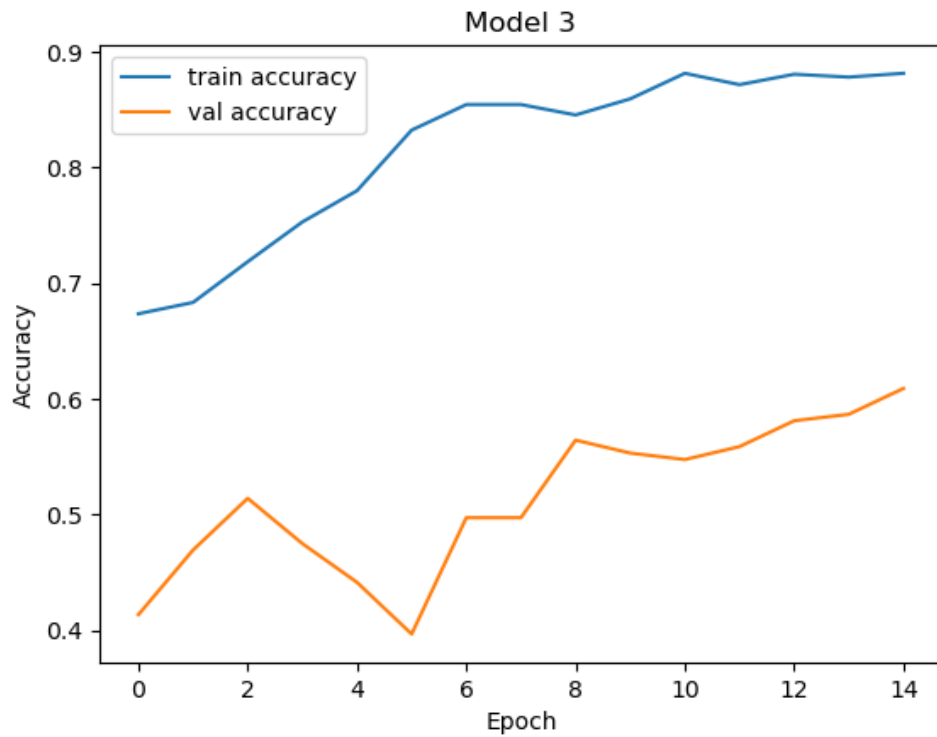


Figure 2.2 Model 3 Validation Curve

ROC Curve for Each Model

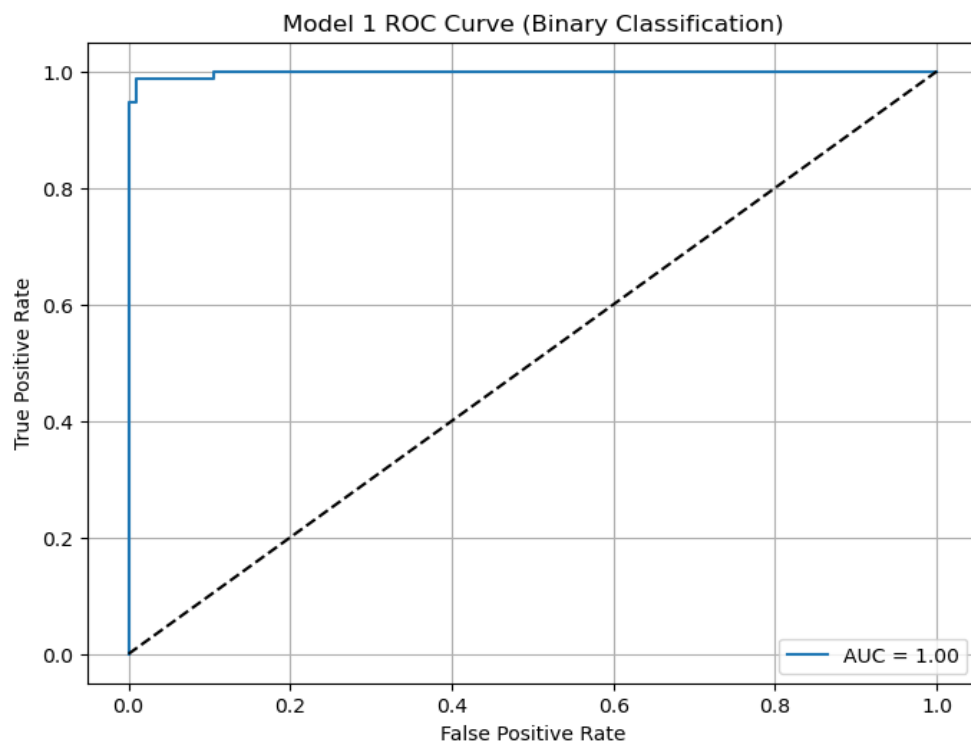


Figure 3. Model 1 ROC Curve

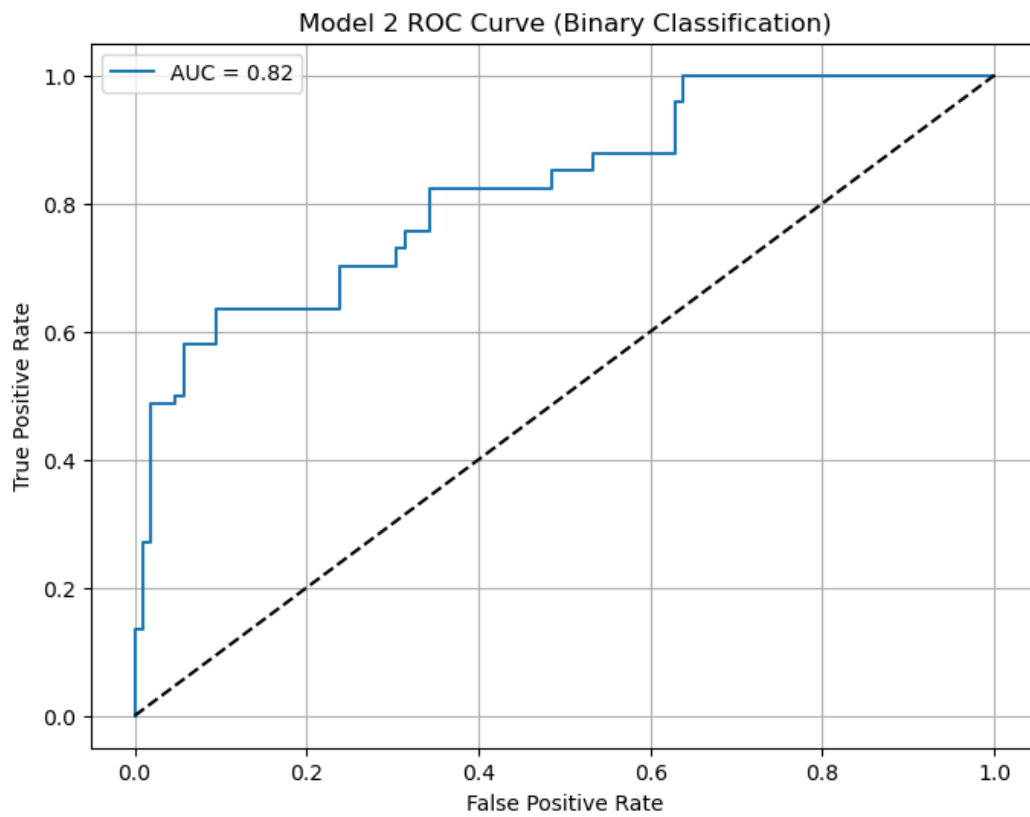


Figure 3.1 Model 2 ROC Curve

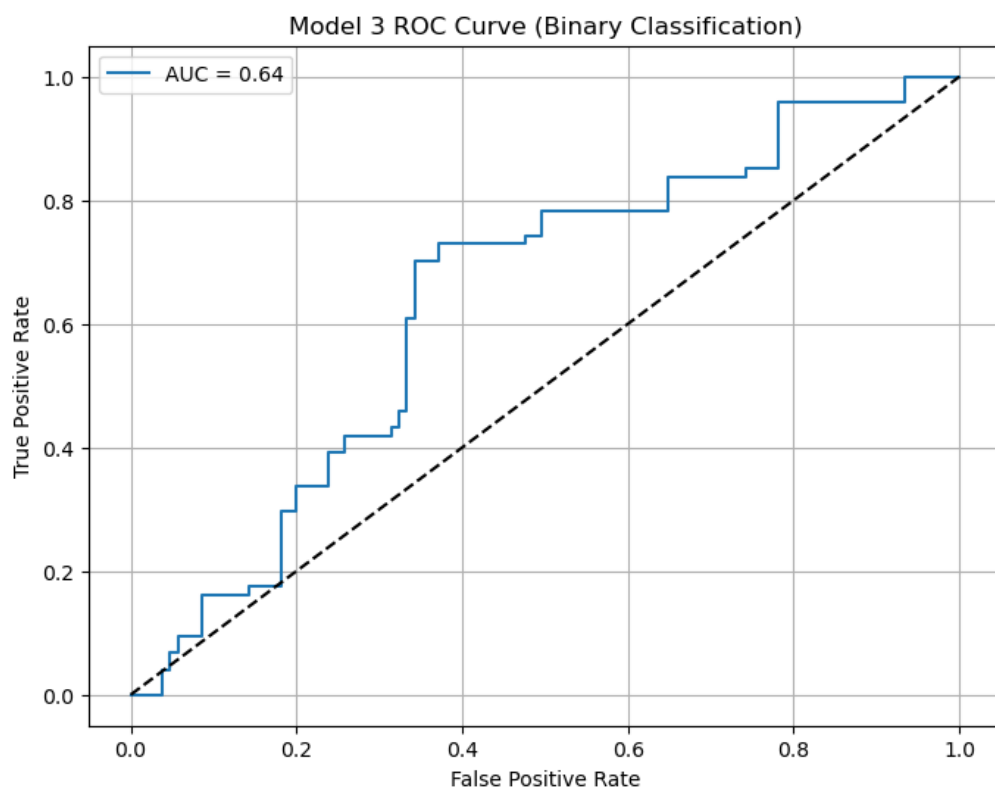


Figure 3.2 Model 3 ROC Curve

Comparative Analysis

The three models differ primarily in depth and preprocessing. Model 1 is a simple baseline CNN, Model 2 introduces an additional convolutional layer and heavy augmentation, while Model 3 is a deeper architecture with mild augmentation.

Model 1 achieved near-perfect training accuracy, while Model 2 and 3 showed slightly lower training accuracy due to increased complexity and the regularizing effects of intensified augmentation. Model 3, despite being deeper, overfit the limited dataset as evidenced by the gap between training and validation performance.

The validation performance indicated that the simpler baseline CNN (Model 1) generalized best on unseen data, achieving higher validation accuracy and AUC than the deeper models. The heavy augmentation in Model 2 reduced overfitting but slightly lowered discriminative performance, while Model 3's depth led to poor generalization due to insufficient data for effective learning.

Overall, these results demonstrate that for small medical imaging datasets, simpler architectures with minimal augmentation may outperform deeper networks with heavy augmentation.

Inference

The baseline CNN (Model 1) performed best on this dataset, highlighting the effect of dataset size on model complexity. Regularization and augmentation, as shown in Model 2 and Model 3, help prevent overfitting but may reduce performance if the dataset is small. In conclusion, increasing model complexity without sufficient data can negatively impact generalization, highlighting the importance of balancing architecture depth, data preprocessing, and dataset size.

1.8 REFERENCES

- Brindha, P.G., Kavinraj, M., Manivasakam, P. and Prasanth, P., 2021, February. Brain tumor detection from MRI images using deep learning techniques. In IOP conference series: materials science and engineering (Vol. 1055, No. 1, p. 012115). IOP Publishing.
- Sharma, S., Gupta, S., Gupta, D., Juneja, A., Khatter, H., Malik, S. and Bitsue, Z.K., 2022. Deep Learning Model for Automatic Classification and Prediction of Brain Tumor. Journal of Sensors, 2022
- Brindha, P.G., Kavinraj, M., Manivasakam, P. and Prasanth, P., 2021, February. Brain tumor detection from MRI images using deep learning techniques. In IOP conference series: materials science and engineering (Vol. 1055, No. 1, p. 012115). IOP Publishing.
- H. E. M. Abdalla and M. Y. Esmail, "Brain Tumor Detection by using Artificial Neural Network," 2018 International Conference on Computer, Control, Electrical, and Electronics Engineering (ICCCEEE), Khartoum, Sudan, 2018, pp. 1-6, doi: 10.1109/ICCCEEE.2018.8515763.
- Hossain, T., Shishir, F.S., Ashraf, M., Al Nasim, M.A. and Shah, F.M., 2019, May. Brain tumor detection using convolutional neural network. In 2019, 1st International Conference on Advances in Science, engineering and Robotics Technology (ICASERT) (pp. 1-6). IEEE.